CASE PRESENTATION

SYEDA SANIYA
3rd YEAR, BGS GIMS

DEMOGRAPHIC DETAILS:

- Name : Mr XYZ
- Age : 43 years
- Sex : Male
- Occupation : Teacher
- Address : Kengeri
- Socioeconomic status : Upper middle class (According to Modified Kuppuswamy scale)

CHIEF COMPLAINTS

- Multiple irregular swellings in the right lower limb since 3 years.
- Pain in the right lower limb since 1 year.
- Pigmentation of skin over right ankle since 3 months.

HISTORY OF PRESENTING ILLNESS

Patient was apparently well 3 years back when he noticed multiple irregular swellings on the inner aspect of the right lower limb. They were insidious in onset and gradually progressive in nature. They increase in size on prolonged standing or walking and decrease on lying down.

They are associated with intermittent episodes of dull aching pain since 1 year in the right calf and the lower leg, aggravates on prolonged standing and walking and relieves on lying down, not associated with night cramps.

The patient also noticed pigmentation of skin over the right ankle 3 months back, which was insidious in onset, gradually progressive, not associated with itching.

- No h/o edema of the leg.
- No h/o fever.
- No h/o abdominal distension, pain and chronic constipation.
- No h/o breathlessness, cough, hemoptysis.
- No h/o trauma, prolonged immobilisation of the limb.
- No h/o non healing wound in the lower limb.
- No h/o previous surgery for varicose veins.
- No h/o long term hospitalisation.

PAST HISTORY

- No h/o similar complaints in the past.
- No h/o diabetes mellitus, hypertension, asthma, epilepsy, tuberculosis.

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TREATMENT HISTORY

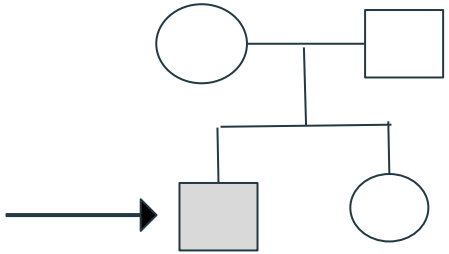
He has been using compression stockings for the past 3 years.

PERSONAL HISTORY

- Diet: Mixed
- Appetite: normal
- Sleep: Adequate
- Bladder habits: regular
- Bowel habits: regular
- No addictive habits

FAMILY HISTORY

No h/o similar complaints in the family.



SUMMARY

A 43 years old gentleman, teacher by occupation presented with complaints of multiple irregular swellings on his right lower limb since 3 years which were insidious in onset, gradually progressive, increase in size on prolonged standing and walking and decreases by lying down, associated with intermittent episodes of dull aching pain since 1 year. He noticed pigmentation of skin over right ankle 3 months back. There is no history of major ailment in the past and there is no history suggestive of deep vein thrombosis.

From the history, I can conclude this is probably a case of varicose veins involving the great saphenous vein.



PHYSICAL EXAMINATION

Informed consent was taken from the patient.

Patient was examined in a well lit room.

Height=170 cm

Weight=90 kg

BMI=31 kg/m^2

VITALS:

PULSE: 75 bpm,regular rhythm, good volume, normal character.

BP: 122/80 mmHg, left arm in sitting position

RESPIRATORY RATE: 15 breaths per minute

TEMPERATURE: 37.5℃

No pallor, icterus, cyanosis, clubbing,
generalised lymphadenopathy, pedal edema.

LOCAL EXAMINATION

Patient was examined in standing position with both limbs exposed from umbilicus below.

INSPECTION:

- Attitude of the limb is normal.
- Localised multiple dilated veins present on his right lower limb : lower 1/3rd of medial aspect of thigh,medial aspect of the leg just below the knee and in front of the medial malleolus.
- Blackish discoloration of the skin over right ankle.
- The skin is not stretched and shiny, no eczema.
- No ulcer, scar, scratch marks, loss of hair.
- Saphena varix is not present.
- The other limb appears to be normal.
- In supine position, the veins are collapsed.

PALPATION

- No local rise of temperature over the dilated veins.
- No tenderness.
- No thickening of veins
- No thickening of skin
- No pitting edema

BRODIE- TRENDELENBURG TEST:

PART 1: Saphenofemoral incompetence present.

PART 2: Perforator incompetence present.

TOURNIQUETS TEST: Incompetence of above knee,
below knee and lower leg perforators**(Dodd,Boyd,Cockett)**

SCHWARTZ TEST: Impulse felt at the saphenous opening-implies valves along GSV are incompetent and involves single continuous column of blood.

PRATT'S TEST: Blow outs seen at- above knee, below knee and lower leg perforator sites.

MORRISSEY'S COUGH IMPULSE: Expansile cough impulse felt at SFJ.

FEGAN'S METHOD: Depressions felt at-above knee, below knee and lower leg perforator sites.

PERCUSSION-Schwartz sign is positive

AUSCULTATION-No murmurs heard

No palpable regional lymph nodes.

Left lower limb is normal.

All arterial pulsations felt.

SYSTEMIC EXAMINATION

CARDIOVASCULAR SYSTEM: S1 S2 heard , no murmurs

RESPIRATORY SYSTEM: Normal vesicular breath sounds heard

CENTRAL NERVOUS SYSTEM: No focal neurological deficits

PER ABDOMEN: No distension,no organomegaly, no palpable mass,bowel sounds heard

PROVISIONAL DIAGNOSIS

Varicose veins in the right lower lower limb of primary etiology, involving great saphenous vein with saphenofemoral incompetence and above knee, below knee and lower leg perforator incompetence without any complications.

CEAP classification: C4aEpAspPr

INVESTIGATION

Duplex USG

OTHER TESTS:

- Complete blood count
- Random blood sugar
- Renal function test
- Liver function test
- ECG
- USG

TREATMENT

Saphenofemoral flush ligation, stripping of great saphenous vein up to the upper calf and ligation of above knee, below knee and lower leg perforators.

THANK YOU